THE STATE

Versus

AHMEND MLANZI

IN THE HIGH COURT OF ZIMBABWE
MUTEVEDZI J
BULAWAYO, 16 JUNE 2025

Criminal Review

MUTEVEDZI J: This matter was placed before me on automatic review in terms of section 57 of the Magistrate Court Act [*Chapter 7:10*] ("the MCA").

[1] The accused person was arraigned before the court of a Provincial Magistrate at Gwanda. He was charged with the crime of malicious damage to property as defined in section 140 of the Criminal Law (Codification and Reform) Act [Chapter 9:23] ("the CODE"). It was alleged that on the 3 April 2024 at house number 747 Geneva, Gwanda, the accused had damaged the windscreen and wipers of a car which belonged to the complainant's car when he threw stones at the vehicle. The matter was reported to the police resulting in the accused's arrest. The value of the damage was estimated at US$150.00.

[2] At the commencement of his trial, the accused pleaded not guilty to the charge. Soon after the accused's plea his counsel went into overdrive and made submissions that the accused lacked the intention to commit the offence because when he did so, he was mentally disordered. In the same address he beseeched the court to dispense with the reading of the state outline because as he put it "the accused would not appreciate the contents." On his part, the prosecutor immediately applied to tender the certificate by which the Prosecutor-General had authorized the prosecution of the accused person after he had recovered from his mental illness. He also tendered a report by a psychiatrist who had examined the accused. In addition, the prosecutor, with the consent of the defence, submitted a statement of agreed facts. Thereafter, he asked the court to return a special verdict of not guilty because of insanity. The court dutifully followed the request and decided as follows:

"Verdict- special verdict of not guilty because of insanity."

[3] The above proceedings demonstrate a worrying lack of understanding of the procedures which must be followed where an accused is alleged to have been mentally challenged or intellectually disordered in one form or another. Those procedures are generally provided for from section 26 to section 30 of the Mental Health Act [chapter 12:15]. For purposes of this review however, the critical issues fall in sections 28 and 29. What a court ought to do is dependent on the state of mind of the accused at the time of the trial and at the time that he allegedly committed the crime in question.

[4] Of significance is that s 28 deals with the procedure which must be followed where an accused person is found to be mentally disordered or intellectually handicapped during trial. It is framed in the following terms:

[5] s 28 of the Act is couched as follows:

> **28 Procedure where person found mentally disordered or intellectually handicapped during preparatory examination or trial**
>
> (2) If, at any time during a preparatory examination against, or trial of, any person—
>
> (a) it appears to the judge or magistrate presiding at the preparatory examination or trial that the person is mentally disordered or intellectually handicapped; or
>
> (b) a report in terms of the proviso to subsection (1) of section twenty-seven is received by the judge or magistrate presiding at the preparatory examination or trial that the person appears to be mentally disordered or intellectually handicapped;
>
> the judge or magistrate shall inquire into that person's mental state.
>
> (3) For the purposes of inquiring into a person's mental state in terms of subsection (2), the judge or magistrate may—
>
> (a) adjourn the preparatory examination or trial for such period, not exceeding fourteen days, as he thinks fit and remand the person to the custody of the officer in charge of a prison; and (b) direct two medical practitioners to examine the person and inquire into his mental state and, after such examination, the medical practitioners shall certify in writing in the prescribed form as to the person's mental state:
>
> Provided that if only one medical practitioner is available, the judge or magistrate may direct a psychiatric nurse practitioner to examine the person concerned, inquire into his mental state and provide the second certificate required by this paragraph."

[10] Although the above provision appears to speak to preparatory examinations, it goes without saying that the use of such procedure was long discontinued in this jurisdiction. As such s 28(2) must be understood to be dealing with accused persons who appear mentally disordered or intellectually handicapped during trial only. It is intended to address instances

where before or during trial, there is a suspicion or danger that the accused person is mentally disordered or intellectually handicapped. Clearly, the objective is to ensure that a court does not expend its energy by trying a person who because of some mental affliction, does not understand what is happening. In addition, because of the centrality of the fair trial rights in our law, as envisioned in s 70 of the Constitution of Zimbabwe, 2013, that person is clearly incapable of properly conducting his/her defence. If that is ascertained, the trial cannot proceed. Instead, the court is required to summarily commit the accused person to a mental institution for treatment. S 28(2) allows a judicial officer to conduct **an inquiry** into the mental state of the accused. That inquiry is carried out by adjourning the trial for a period not in excess of 14 days during which the accused is remanded into the custody of the officer in charge of a prison. Thereafter, the court makes an order directing that two medical practitioners (one of whom may be a psychiatric nurse) must examine and inquire into the accused's mental state and set out their findings in writing in the prescribed form. If and when the medical practitioners have certified that the accused is mentally disordered or intellectually handicapped their reports must specifically make the recommendation on whether the accused **is or is not fit to stand trial.**

[6] I refer to the above provisions because of what appears in the record of proceedings. As already mentioned, counsel for the accused specially indicated to the court that the accused was mentally disordered to the extent that he could not understand what was contained in the outline of the state case. If that was the case, there was therefore no need for the proceedings that followed. The court could not have possibly undertaken a trial in which the accused understood nothing. The accused could not have pleaded not guilty to a charge which he did not understand. A trial is not for the prosecutor and the legal practitioner. Instead, it is against the accused. A legal practitioner can only represent the accused on that accused's instructions. He/she cannot come to court and do as they please without the instructions of the person they represent. The court ought to have queried the representation that the accused was mentally disordered at the time these proceedings were carried out. It unfortunately did not. If it were true, which I doubt it was, then the correct procedure would have been to resort to s 28(2) of the MHA by which the trial would have been stopped and the accused referred for examination by two medical practitioners. If they certified the

mental disorder, the court would have then committed the accused to e mental institution for treatment. Where the doctors certify that the person is not mentally disordered or intellectually handicapped and is fit to stand trial, the trial of the accused must proceed as if there had never been any suspicion about his mental capacity.

[7] On its part section 29 deals with instances where an accused is alleged to have been mentally disordered or intellectually handicapped at the time he/she allegedly committed the crime. It states that: -

> "**29 Procedure where person charged is found mentally disordered or intellectually handicapped at time of committing offence**
> (2) If a judge or magistrate presiding over a criminal trial is satisfied from evidence, including medical evidence, given at the trial that the accused person did the act constituting the offence charged or any other offence of which he may be convicted on the charge, but that when he did the act he was mentally disordered or intellectually handicapped so as to have a complete defence in terms of section 248 of the Criminal Law Code, the judge or magistrate shall return a special verdict to the effect that the accused person is not guilty because of insanity" (Underlining is my emphasis).

[8] When an accused seeks to rely on s 29(2), he/she is basically alleging that at the time he allegedly committed the offence, he/she was suffering from a mental defect. In other words, the accused will be contending that he has fully recovered from that illness and is able to undergo trial. If the allegation remains that he was mentally ill at the material time and has still not recovered from that problem, the procedure under s 28(2) would cover him/her. Put in yet another way, s 29(2) applies where the accused has sufficiently recovered from mental illness and is fit to stand trial but his defence is that he was mentally disordered at the time of commission of the offence.

[9] The procedure under s 29(2) is therefore not a summary procedure. Rather, it is an elaborate formula which must be followed during a trial. The provision is a defence which must be pleaded at trial. Once that is admitted, it follows that where an accused person wishes to depend on it, the court must undertake a trial in one way or another. The rituals which are observed at any other criminal trial must be followed. The charge must be read to the accused who must plead not guilty to it. The outline of prosecution's case is equally read. The accused then gives his defence outline. It is in that outline that he/she must state that he was mentally disordered or intellectually handicapped at the time of commission of the crime and could not have been responsible for his/her actions. Put bluntly, the accused is

raising a defence just like one could do with self-defense, intoxication, provocation or some other. The law is that the court must be satisfied from evidence including medical evidence, that the accused was not responsible for his actions at the material time because of a mental affliction. What that means is that a bald allegation that the accused was mentally challenged at the material time would not suffice. It must be supported by evidence from the usual sources of evidence and evidence from a medical practitioner. An accused who comes to court without medical records to support him/her risks having his/her defence thrown out.

[10] A trial may be conducted in various ways. It is not only conducted where the prosecution and the defence have disagreements but also includes instances where the state may be accepting that the accused was mentally disordered at the material time. That acceptance does not remove the requirement for the adduction of evidence including medical evidence. That can be achieved in different ways. For instance, the prosecutor may tender affidavits from medical practitioners. He may also seek formal admissions of the testimonies of witnesses in terms of s 314 of the Criminal Procedure and Evidence Act [chapter 9:07] (the CPEA) or may like what happened in this case, draw a statement of agreed facts with the defence. Such course would in my view, still amount to conducting the trail as required by law.

[11] In instances where the state disagrees with the contention that the accused was mentally disordered at the material time, a normal trial must be undertaken. The state calls its witnesses and presents exhibits. See the case of *S v Fungayi Chiwaya* HH-821-22. Common-sense will dictate that where the prosecution disputes the insanity of an accused person, it cannot be their duty to procure evidence which shows that the accused was mentally disordered at the material time. The accused is required to prove, on a balance of probabilities, that he was mentally ill at the relevant time. The defence of mental disorder at the time of commission of the crime is born out of the common-law defence of insanity. Its operation is guided by section 29 of MHA which prescribes the procedure which courts must adhere to and Part V of the Criminal Law Code which creates the substantive defence. Part V provides that:

MENTAL DISORDER

226 Interpretation in Part V of Chapter XIV
In this Part–
"mental disorder or defect" means mental illness, arrested or incomplete development of mind, psycho-pathic disorder or any other disorder or disability of the mind.
227 Mental disorder at time of commission of crime
(1) The fact that a person charged with a crime was suffering from a mental disorder or defect when the person did or omitted to do anything which is an essential element of the crime charged shall be a complete defence to the charge if the mental disorder or defect made him or her–
(a) incapable of appreciating the nature of his or her conduct, or that his or her conduct was unlawful, or both; or
(b) incapable, notwithstanding that he or she appreciated the nature of his or her conduct, or that his or her conduct was unlawful, or both, of acting in accordance with such an appreciation.
(2) For the purposes of subsection (1), the cause and duration of the mental disorder or defect shall be immaterial.
(3) Subsection (1) shall not apply to a mental disorder or defect which is neither permanent nor long-lasting, suffered by a person as a result of voluntary intoxication as defined in section two hundred and nine-teen."

[12] The normal trial procedures such as cross examination and reexamination of witnesses happen. The defence then presents its own case but bearing in mind that it is the accused who has the responsibility to prove, on a balance of probabilities that he was mentally disordered at the relevant time. The procedure under s 29(2) must be appreciated from the background that it ends with a verdict unlike under section 28. The accused must either be found guilty or not guilty because of insanity. Such verdict cannot be arrived at without a trial being undertaken. See the cases of *S v Emelda Marazani* HH-192-23; and *S v Godfrey Karomo* HH546/22

[13] In this case, my worry was not about the verdict because it was correct to hold that the accused was not guilty because of insanity. The apprehension I have stemmed from the conduct of the accused person's trial if there was any. It was carried out in a haphazard manner. The procedures were not properly adhered to. The accused did not present a defence outline. The court did not give reasons why it arrived at the verdict of not guilty because of insanity which it returned at the end. In addition, the statement of agreed facts has no other evidence, which showed that the accused was mentally challenged at the relevant time, apart from mentioning the medical report. In the case of *S* v *Gambanga* SC32/98 it stated that it should be borne in mind that medical reports suggesting that a person may have been suffering from a mental illness at the time of the commission of the

offence need to be supported by some other evidence. On their own, such reports may not be conclusive. That was clearly omitted in this case. Further, the verdict itself was not properly phrased. There was no need for the court to state **"Verdict- special verdict of not guilty because of insanity."** Instead, the court must simply state that the accused is not guilty because of insanity. Once that is stated, it becomes obvious that the court will have returned a special verdict.

[14] A reading of the record of proceedings shows an affidavit deposed to by one Dr. Elena Poskotchinova, a psychiatrist stationed at Ingutsheni Central Hospital, Bulawayo. It was marked as Exhibit 2. In paragraphs 4 to 6 of the same, the Psychiatrist stated: -

> "4. When examined by me, the accused who is 26 years old, street resident appeared to be unkempt and very psychotic with incoherent speech, blunted affect and loose thought association. He denied his crime with paralogical explanation. "it was bomb inside of car, why l tried to protect explosion." lack of insight. He underwent long course of treatment settled down and explained that he is a known psychiatric patient with 4 admissions at Ingutsheni Hospital in 2017, last admission in 2023 with diagnosis of Schizophrenia. Defaulted treatment and relapsed in 2024. Denied alcohol or any drug misuse. There is no available history of mental disorder due to poor family support.
> <u>According to staff report</u>
> Despite treatment, still isolated, withdrawn, poor self-care.
> 5.<u>In my opinion:-</u> there is a reasonable possibility that at the time of the alleged crime the accused was suffering from mental disorder Chronic Schizophrenia. He was mentally disturbed to such an extent that he should not be held legally responsible for his action. He is dangerous to society (street resident) and needs special verdict to return,
> 6.He is fit to stand trial."

[15] The above report leaves me in no doubt that the accused may have been suffering from a mental affliction at the time that he committed the crime. It is for that reason that I choose not to vitiate the proceedings. It would not serve any purpose because ultimately any court before which the accused may be tried will arrive at the same conclusion.

[16] From the above, it is apparent that the trial magistrate fell into error. He did not conduct a proper trial. Relying only on the medical report and submissions by counsel was not sufficient. Clearly, these proceedings were not in accordance with real and substantial justice but for reasons that I have already stated, I am hamstrung to do anything other than withholding my certificate which I hereby do.

MUTEVEDZI J…….